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Name: عدليه فتحي عبدالعليم مراد National ID: 26809012104067	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM 25/9/2025 G

Name	عبد الله محمد	MRN	٤٠١٤٠٩٠٩٠٩٠٩
Age	57yo	ID	1970/12/05
Menopausal status	post	Phone	0110023880
NRS SCORE		Marital status	widow
BMI		Family history	-ve
CO morbidity	M → H/L	REASON OF MDT	New

Clinical Data

BX: Left Ix II

ER:	PR:	HER 2:	Ki67:
818	818	-ve	5%

R

Sonomammography: ACIRB

100% isodense Masses suspicious
Microcalcifications 6.7 x 3.8 cm.

Mets Workup:

away from N By cystic TNM
lesion 1 x 0.6 cm.

Normal skin

LA level 1

Recommendations:

5m 0.9 x 0.6 cm e' lost tubes.

Benign UIC Calcification

CAP → No evidence

after Reviewing Sm films

for possibility of N/ACT

MDT OUTCOME & PLAN

for up front 4 MRM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. ن. م. م.	د. م. م. م.	د. م. م. م.			



الجمهورية العربية السورية
الوزارة العامة للصحة
مديرية الشؤون الصحية
مديرية حلب

قسم الباثولوجي

PATHOLOGY DEPARTMENT HISTOPATHOLOGICAL REPORT

Patient name: عناية فاضل محمد مراد

pending HER2

Request type: عيانية

Hospital no:

Department: مصلحة المرأة

Referred by: د/ امل حجازي

Nature of specimen: Tru-Cut Biopsy

Clinical diagnosis: left breast mass.

Age: 57 years

Sex: Female

Sub request type:

Pathology No.: 2343-2025

Date of Result: 5/7/2025

Receiving date: 24 /6/2025

I.D. Number: 26809012104067

Gross pathology:

Multiple grayish white tissue cores up to 1 cm long, totally submitted.

Immunohistochemical technique:

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine, and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen, and counter-stain. The sections were treated against ER, PR, Her2, Ki67.

Microradiography:

Microradiographic examination revealed malignant growth, formed of cords, clusters, nest and single cells of anaplastic ductal cells with moderate anaplasia and moderate tendency to form tubules, surrounded by desmoplastic stroma with mild lymphocytic response.

Immunohistochemistry:

ER: Positive Strong nuclear staining in 70% of tumor cells (Allred Score 3/3)
PR: Positive strong nuclear staining of 70% of tumor cells (Allred Score 3/3)

HER2/PAM50

Ki67: Positive nuclear staining of 5% of tumor cells.

Diagnosis: left breast mass, Tru-Cut Biopsy:

Invasive Duct Carcinoma (NST), Grade II

Signature:

Dr. Mona Mohamed, M.D

Pathology Lab. Manager

Norhane Nabil, M.D

Patient ID: 84234

Age: 56 سنة 9 شهر

Patient Name English: Adliah Fatehy Abdalattir.

2025/06/17 11:31:23 AM : Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography

Clinical summary: 57 y old lady coming for left breast lump few years ago and increased in size at last 6 months .

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibro glandular densities (ACR B).
- UOQ is the seat of spiculated isodense mass with suspicious micro calcifications it measures 6.7x3.8 cm .
- **Ultrasound** demonstrated a large spiculated heterogenous solid mass lesion with internal high vascularity by color Doppler study , it seen at 2-3 o'clock zone A para-areolar area and measures 6.9x1.9 cm away from the nipple by cystic lesion measures 1 x 0.6 cm .
- Normal skin and nipple/areolar complex.
- Level I two pathological axillary LNs measure 1.6x0.9 cm cortex 5 mm and 0.9x0.5 cm with lost hilum .

Right breast

- Scattered fibro glandular densities (ACR B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications yet UIQ vascular calcifications .
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Preserved axillary LNs.

Impression:

Left breast (BIRADS 5): UOQ suspicious solid mass lesion with two level I pathological axillary L.Ns .

Right breast (BIRADS 2): UIQ benign vascular calcifications .

Recommendations:

For Tru cut biopsy and histopathological correlations .

Dr. Nashwa A. El Kader